

Opportunities for HHS to Accelerate Patient-Facing Clinical AI through Payment Reform

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Response to HHS Request for Information: Accelerating the Adoption and Use of Artificial Intelligence as Part of Clinical Care (RIN 0955-AA13; 90 FR 60108)

Executive Summary

Patient-facing clinical AI represents a rare opportunity to introduce new technology that has potential to reduce healthcare spending while improving access. The primary barrier to scaling these gains system-wide is ambiguity in payment pathways (Q1, Q7). Today, no billing codes exist for AI-enabled clinical services, asynchronous care remains largely unreimbursable, and each payer relationship requires bespoke negotiation that cannot scale.

This response reflects the operational experience of leaders at five companies delivering AI-enabled clinical care across pediatrics, HIV/STI care, primary care, and chronic disease management: Summer Health, Healthvana, Counsel Health, August AI, and Doctronic.¹ We represent an emerging market category – patient-facing clinical AI – and draw on direct operational, real-world experience to address the RFI's questions on barriers to adoption (Q1), payment policy changes (Q2), where AI tools have met or exceeded expectations (Q6), and administrative hurdles to adoption (Q7).

We recommend a phased approach (Q1, Q2, Q7):²

- **Near-term:** Modernize fee-for-service by recognizing asynchronous message-based care as a billable modality nationwide (42 CFR § 410.78), creating an AI-augmented modifier for existing E/M codes (42 CFR § 414.22), and expanding APCM codes to cover AI-enabled care (G0556-G0558; 89 FR 97710)
- **Medium-term:** Establish billing codes for AI-autonomous clinical services (42 CFR § 410.32) and pilot PMPM models through CMMI
- **Long-term:** Layer outcomes-based payment as the claims data and clinical evidence generated by earlier phases mature

These recommendations are intended to be net-deflationary, where each new payment pathway reimburses AI-enabled care at rates below traditional equivalents, capturing cost savings while

¹ Descriptions of each company represented in this response are provided in Appendix 1.

² A summary of all recommendations and corresponding regulatory citations is provided in Appendix 2.

expanding access. HHS has an opportunity to channel this innovation toward achieving Triple Aim results rather than ceding it to unregulated alternatives.

Patient-Facing Clinical AI Delivers Clinical, Access, Operational, and Cost Outcomes

AI is performing real clinical work: diagnosis, treatment guidance, chronic disease management, and more. From our combined operations across pediatrics, HIV/STI care, primary care, and chronic disease management, we've achieved the following outcomes (Q6):

Cost:

- \$10/visit target at scale, down from current models (Summer Health).
- \$29 visits, already below typical insurance video visit rates (Counsel Health).
- \$39 cash visits with AI-generated documentation matching chief resident quality (Doctronic).

Improved access:

- 13,000 previously unanswered patient messages are now resulting in clinical guidance (Healthvana).
- 24/7 coverage with response times under 15 minutes (Summer Health) and under 10 minutes (Counsel Health), with 4-5x the patient capacity of traditional video telemedicine without sacrificing response quality (Counsel Health).

Clinical quality:

- Nearly 2x medication persistence and 3x PrEP initiation among patients using AI-powered HIV prevention support (Healthvana), vs. national baselines of >50% first-year discontinuation³ and only 25% prescription rate among those with a PrEP indication.⁴
- 100% of patient conversations evaluated for quality in near real-time, compared to less than 1% in traditional retrospective chart review – enabling a level of auditability and transparency that exceeds traditional quality oversight (Counsel Health).
- Diagnostic accuracy that catches physician errors, serving as a clinical decision support layer (August AI).

Operational efficiency:

- 7 visits/hour today, targeting 10-12 as AI handles more of the clinical workflow (Summer Health).
- 50-70% of visit time (primarily data exchange and intake) offloaded to AI before the physician encounter (Doctronic).

³ McCormick CD, Sullivan PS, Qato DM, Crawford SY, Schumock GT, Lee TA. Adherence and persistence of HIV pre-exposure prophylaxis use in the United States. *Pharmacoepidemiol Drug Saf.* 2024;33(1):e5729.

⁴ CDC. 2020 HIV Surveillance Report. National Center for HIV, Viral Hepatitis, STD, and Tuberculosis Prevention. 2024. Available at: <https://www.cdc.gov/nchhstp/director-letters/2020-hiv-surveillance-report.html>.

- 56% of clinical staff saving 5+ hours/week, redirecting time to complex cases (Healthvana).
- 2/3 of patient chats resolved AI-only; 1/3 escalated to a physician, demonstrating safe triage (Counsel Health).

These results come from live clinical operations serving patients at scale across multiple specialties and geographies.

Current Payment Models Do Not Recognize AI-Enabled Care

The payment system that HHS aims to reform was built on a foundational assumption: a clinician delivers a service to a patient in a discrete encounter. Payment was designed for brick-and-mortar health systems that provided episodic care in an era when acute care was more important and prevalent than chronic care management.

As care needs and patient preferences have evolved, payment mechanisms for patient-facing clinical AI have not kept up. AI-enabled care breaks every element of that assumption: the clinician may be supervising rather than personally delivering care, the encounter may be asynchronous or continuous rather than discrete, and the value may show up months later in avoided ER visits rather than in a single billable moment. While a handful of procedure-specific CPT codes exist for narrowly scoped AI diagnostics (e.g., CPT 92229 for diabetic retinopathy screening), there is no scalable national coding pathway for patient-facing AI that delivers triage, treatment guidance, or ongoing care management.

Today, patient-facing clinical AI companies survive through a patchwork: cash-pay visits (Summer Health, Counsel Health, Doctronic), employer contracts (Counsel Health), 340B drug pricing savings (Healthvana), or direct-to-consumer subscription (Summer Health, Counsel Health, Doctronic, August AI). Autonomous AI care (live in Utah through Doctronic) has no payment pathway; only the human visit component is billable.

The specific barriers we face (Q1, Q7):

- **Asynchronous care is not recognized as a billable modality.** CMS and most state Medicaid programs do not classify fully asynchronous text-based encounters as reimbursable, despite widespread adoption of message-based care by providers managing growing patient panels. Summer Health is the only company among us to have secured payer reimbursement for asynchronous care, a process that took three years of negotiation.
- **No billing codes exist for AI-enabled clinical services.** There is no CPT modifier or service code for AI-assisted or AI-delivered care. When AI is deployed within health systems today, it generates no direct reimbursement. Any value captured flows through staffing efficiency or cross-subsidized programs, not through the payment system itself.
- **Current contracting models require bespoke negotiation that cannot scale.** PMPM or case-rate arrangements must be negotiated payer by payer, plan by plan, which creates uncertainty on both sides of the transaction: AI companies cannot reliably

forecast revenue, and health system buyers cannot justify procurement without a billing code to point to.

As a result, innovation gets bottlenecked into direct-to-consumer approaches instead of scaling through the healthcare system (Q1). Without clear reimbursement pathways, adoption will remain confined to cash-pay patients and bespoke payer arrangements, limiting the market to companies that can survive these conditions rather than those delivering the best care.

Clear payment pathways would transform AI-enabled care from a niche product into a deflationary force for the whole system, lowering costs and broadening access beyond the limited markets where this technology is already proving its value.

HHS Can Design Payment Policy That Improves Care and Lowers the Total Cost of Care

HHS is right to be concerned that covering innovations rarely reduces net spending. The history of healthcare payment is full of spending bubbles that followed new billing codes. To realize the deflationary potential of AI-enabled care, payment systems must be designed to capture those gains rather than repeat past patterns.

This requires learning from prior payment experience in both directions. Underpayment kills adoption: one cost analysis of the Medicare Diabetes Prevention Program found that delivering the program cost \$553 per participant while Medicare only reimbursed \$108, and provider participation remained limited nationwide.⁵ A similar dynamic played out with IDx-DR, the first autonomous AI diagnostic to receive a dedicated CPT code (92229) for diabetic retinopathy screening. Despite FDA clearance and strong clinical evidence, adoption was slower than expected in part because billing complexity and workflow integration costs outweighed available reimbursement.⁶ Conversely, loosely defined codes invite overuse: the rapid proliferation of remote patient monitoring billing, where broad code definitions led to widespread abusive billing and subsequent enforcement action, illustrates the opposite failure mode.⁷

Our recommendation for episode-based bundling is designed to avoid both extremes. Across all of these mechanisms, payment design should focus on per-episode reimbursement rather than per-interaction billing, ensuring that cost savings accrue to the system rather than being eroded by volume inflation. A unified national framework also would prevent the state-by-state

⁵ Parsons AS, Raman V, Starr B, Zezza M, Rehm CD. Medicare underpayment for Diabetes Prevention Program: implications for DPP suppliers. *Am J Manag Care*. 2018;24(10):475-478.

⁶ Rajesh AE, Davidson OQ, Lee CS, Lee AY. Artificial intelligence and diabetic retinopathy: AI framework, prospective studies, head-to-head validation, and cost-effectiveness. *Diabetes Care*. 2023;46(10):1728-1739.

⁷ Office of Inspector General, U.S. Department of Health and Human Services. *Billing for Remote Patient Monitoring in Medicare*. August 2025.

fragmentation that slowed telehealth adoption, where inconsistent reimbursement policies across state Medicaid programs created years of uncertainty for providers and patients alike.⁸

Though we recognize HHS's interest in outcomes-based payment, value-based care is the wrong starting point for a nascent technology category, since it requires knowing how a technology creates value, measuring it reliably, and attributing outcomes – none of which are mature for clinical AI today. Starting with VBC would slow adoption and concentrate gains among sophisticated players who can afford actuarial modeling and complex billing infrastructure, rather than rewarding those who deliver the best care.

Fee-for-service provides the predictable revenue environment that health systems and AI companies need to de-risk investment. It also creates the billing infrastructure and data capture necessary to eventually measure outcomes at scale. Without a foundation of reimbursable AI encounters, there is no data substrate for outcomes-based payment to build on. Given this, we propose the following phased approach (Q2).

Near-term: Modernize fee-for-service for AI-enabled care (Q2)

Recognize asynchronous care as a billable modality.

We recommend that CMS amend 42 CFR § 410.78 to recognize asynchronous message-based clinical encounters as reimbursable nationwide. Current regulations limit store-and-forward technologies to Alaska and Hawaii demonstration programs (42 CFR § 410.78(d)), despite widespread clinical adoption of text-based care. This limitation is particularly consequential for rural communities, where text-based asynchronous care can be delivered over cellular networks without requiring reliable broadband, specialist proximity, or scheduling around limited clinic hours. The geographic-access rationale that justified the Alaska and Hawaii demonstrations applies nationwide.

To prevent inflationary billing, we recommend episode-based bundling rather than per-message reimbursement. All messages and AI interactions related to the same clinical issue would be bundled into a single billable episode over a fixed time window (e.g., 7 days), consistent with existing asynchronous care codes. This structure allows patients to return repeatedly over several days (AI-enabled care is always on), while providers bill once per episode rather than per interaction. This avoids the obvious abuse case where marginal AI touches generate repeated charges.

Research on telehealth utilization supports this design: unbundled per-interaction billing has been shown to increase spending without corresponding improvements in outcomes, while

⁸ Center for Connected Health Policy. *State Telehealth Laws and Reimbursement Policies Report, Fall 2024*. 2024. Available at: <https://www.cchpca.org/resources/state-telehealth-laws-and-reimbursement-policies-report-fall-2024/>.

episode-based structures contain costs by aligning reimbursement with clinical resolution rather than message volume.⁹

We further recommend removing the requirement that an initial visit be conducted via video or in-person before asynchronous follow-up is billable. This requirement was designed to establish a patient-provider relationship before asynchronous follow-up, but it blocks AI-native care models that begin with text-based triage. We propose replacing it with direct safeguards that address the same concern more effectively. Each billable async episode must include a defined clinical question, a documented clinical assessment, and a treatment recommendation or an escalation decision.

Create an AI-augmented care modifier for existing E/M codes.

We recommend CMS establish an AI-augmented modifier applicable to existing evaluation and management codes, similar to the -93 modifier for audio-only services (42 CFR § 414.22). This modifier would enable reimbursement at a reduced rate (e.g., 80% of the standard rate), reflecting the lower cost of delivering AI-assisted care. Eligibility should require documentation of AI-assisted components, clinical data capture, defined escalation protocols, and clinician attestation of review.

Current E/M codes carry time requirements. The 99213 CPT code, for example, assumes 15-20 minutes of clinician involvement. When AI handles intake, history-taking, and documentation, the physician's role shrinks to review and decision-making, often 2-3 minutes. Under current rules, this shorter involvement means the visit doesn't meet time thresholds for billing. We recommend that the AI-augmented modifier be tied to the service delivered rather than to the clinician's minutes spent. This avoids both extremes: underpayment for genuinely efficient AI-assisted care, and abusive billing for marginal AI involvement (two seconds of AI interaction should not generate a \$200 charge).

When rates are set correctly, payer incentives align naturally. If AI-augmented visits are reimbursed at a fraction of standard E/M rates and AI-assisted workflows enable a physician to see more patients, total cost per treated patient declines even as encounter volume increases.

Expanding Advanced Primary Care Management codes to cover AI

CMS's introduction of Advanced Primary Care Management (APCM) codes (G0556-G0558; 89 FR 97710) represents meaningful progress toward the kind of payment infrastructure AI-enabled care requires. Several of its 13 service elements (i.e., 24/7 access to care, enhanced communication opportunities, population data analysis) describe capabilities that AI-enabled platforms already deliver as core functionality. CMS's decision to replace time-based thresholds with risk-tiered reimbursement was the right design choice and should be preserved. We propose two specific modifications that would make APCM viable for AI-enabled care models:

⁹ Ashwood JS, Mehrotra A, Cowling D, Uscher-Pines L. Direct-to-consumer telehealth may increase access to care but does not decrease spending. *Health Affairs*. 2017;36(3):485-491.

First, the initiating visit requirement allows telehealth but requires a synchronous encounter for new patients or those not seen by the practice in three years. AI-native platforms onboard patients through asynchronous text-based intake, triage, and clinical assessment, which do not qualify under current rules. CMS should allow documented asynchronous onboarding for APCM and predicates such as TPM, as we recommend for asynchronous care, to satisfy this requirement. TCM's mandatory face-to-face visit within 7-14 days (CPT 99495, 99496) should accommodate asynchronous follow-up.

Second, APCM's attribution model requires a single named clinician to serve as the patient's comprehensive primary care provider. AI-enabled platforms may involve rotating clinical oversight or team-based supervision rather than a single named practitioner, and often deliver condition-specific management (HIV prevention, pediatric triage, chronic disease support) rather than serving as the patient's full medical home. CMS should expand eligibility to include condition-specific care management and clarify that platforms with defined clinical governance and escalation protocols can qualify as billing entities.

Medium-term: Create pathways for AI-delivered care (Q2)

Establish billing codes for AI-autonomous clinical services.

For encounters in which AI delivers clinical services directly (diagnosis, triage, treatment guidance) with defined escalation protocols to human clinicians, we recommend that CMS establish a new service category (42 CFR § 410.32 or a new section). This would provide a payment pathway for fully autonomous AI care, which currently has no billing mechanism. For example, Doctronic operates autonomous AI care in Utah today; only the human visit component is reimbursable.

We recognize that autonomous AI care warrants a risk-tiered approach. Initial billing codes should target lower-acuity, protocol-driven scenarios where diagnostic and treatment pathways are well established, such as routine UTI management, standard lab interpretation, and medication refill assessments. Higher-complexity presentations would require mandatory escalation protocols to human clinicians. As evidence of safety, efficacy, and cost-effectiveness accumulates, the scope of reimbursable autonomous encounters can expand accordingly.

There should be room in the market for reimbursement across the full spectrum from AI-augmented care with clinical oversight to AI-delivered care with human escalation pathways. Both models exist today; neither has a clear payment pathway.

Support CMMI pilots for subscription and PMPM models.

The always-on nature of AI-enabled care lends itself to subscription or per-member-per-month models rather than visit-based billing. A patient may message an AI platform daily for chronic disease support, which does not fit discrete encounter logic. Forcing it into per-visit billing

creates perverse incentives. We recommend CMMI pilot a simple PMPM model for AI-enabled primary care with straightforward attribution, creating an alternative pathway that matches how these tools actually deliver value.

Long-term: Layer outcomes-based payment as evidence matures (Q2)

Attribution problems (i.e., isolating AI's contribution from human care interventions) are real, and current measurement infrastructure cannot support them. Once near- and medium-term fee-for-service codes generate sufficient claims data, outcomes-based models become viable. The earlier phases build the data substrate that value-based care requires.

Accelerating adoption beyond payment reform (Q2, Q4, Q5)

State-level innovation is already underway (Q5). Utah's AI Learning Laboratory Program (in which Doctronic is participating) has enabled autonomous AI care delivery with appropriate oversight requirements.¹⁰ HHS can accelerate this by providing federal payment pathways that reward successful state experimentation rather than requiring each state to build from scratch.

Additionally, the Rural Health Transformation Program (P.L. 119-21, § 71401) allocates \$10 billion annually to states for rural health improvement, and the statute already contemplates technology-enabled care delivery, including remote monitoring and AI. CMS should actively encourage states to incorporate AI-enabled clinical care models into their transformation plans, providing a funded pathway to test what this RFI envisions alongside the payment reforms proposed above.

A note on evidence standards

Current evidence frameworks assume AI introduces risk that must be proven away; however, AI-enabled care can be evaluated for 100% of encounters, compared to the small-sample chart review that characterizes traditional quality assurance. This includes the ability to report outcomes stratified by race, ethnicity, language, and disability status, which is critical for ensuring that AI tools do not distort risk adjustment or produce disparate escalation patterns across populations. HHS should develop evidence standards that account for this inversion in which the AI-enabled model offers greater visibility into care quality, not less.

While payment ambiguity is the primary operational bottleneck, safety oversight and validation standards are equally critical to sustainable adoption. Payment reform should proceed in parallel with risk-tiered regulatory standards and evidence expectations.

¹⁰ Utah Department of Commerce, Office of Artificial Intelligence Policy. *Utah and Doctronic Announce Groundbreaking Partnership for AI Prescription Medication Renewals*. January 6, 2026. Available at: <https://commerce.utah.gov/2026/01/06/news-release-utah-and-doctronic-announce-groundbreaking-partnership-for-ai-prescription-medication-renewals/>

Veterans Affairs, Indian Health Service, CMMI, ARPA-H, and other HHS divisions each offer distinct environments for validating AI-enabled care across diverse demographics, care settings, and conditions. We recommend that HHS partner with these agencies to define what these frameworks should look like, drawing on operational data from early adopters rather than imposing untested requirements that could freeze the market (Q4).

Without Payment Reform, AI-Enabled Care Cannot Scale Through the Healthcare System

The companies represented in this response will continue to operate regardless of HHS action. Sufficient consumer demand exists to sustain direct-to-consumer business models, and entrepreneurs will continue to build around the healthcare system if the system does not adapt. Without HHS action, however, these gains will remain confined to cash-pay patients in metro areas rather than reducing costs and expanding access system-wide, including to rural communities where provider shortages make traditional models unsustainable.

The alternative is a fragmented, adversarial market that serves no stakeholder well. Health systems will deploy AI under legacy fee-for-service rules and use it to dramatically increase visit volume, capturing efficiency gains as revenue growth rather than lower total cost of care. Payers, responding to rising utilization without a clear value or pricing framework, will refuse or restrict reimbursement for AI-enabled encounters. Fully autonomous AI care, which is already operating in limited state sandboxes, will never progress beyond pilots because there is no dedicated, nationally recognized payment pathway to support its adoption.

Meanwhile, patients are already turning to general-purpose consumer AI for health guidance. Platforms like OpenAI's ChatGPT and Anthropic's Claude operate without HIPAA obligations or clinical accountability, yet they are rapidly becoming the *de facto* source of health information for millions of Americans. Consumers no longer need permission to ask about their healthcare. If purpose-built clinical AI cannot scale through the healthcare system, general-purpose LLMs without healthcare-specific obligations will fill the vacuum by default.

HHS has the tools to prevent this outcome. CMS's ACCESS model already points in the right direction: its rates assume technology-enabled efficiency, creating a pathway for scaled digital platforms rather than traditional fee-for-service operations.

Specifically, we recommend that CMS recognize asynchronous care as a billable modality nationwide, create an AI-augmented modifier for existing E/M codes, expand APCM codes to cover AI, establish billing codes for AI-autonomous clinical services, and pilot PMPM models through CMMI. These payment pathways, designed to reimburse AI-enabled care at rates below traditional equivalents, would reduce costs and expand access while generating the claims data needed to support outcomes-based payment over time. We welcome the opportunity to work with HHS to design and pilot these pathways.

Appendix 1: Companies represented in this response

- [August AI](#) is an AI health companion used by 6M+ people across 160 countries to interpret lab reports, understand symptoms, and get personalized health guidance via WhatsApp.
- [Doctronic](#) is a multi-agent AI health platform that connects patients to personalized guidance and licensed doctors in all 50 states.
- [Summer Health](#) is a text-based pediatric care platform that connects parents to licensed pediatricians within 15 minutes, 24/7, for urgent care, developmental questions, and prescriptions.
- [Healthvana](#) is an AI-powered telehealth platform for HIV prevention and sexual health. Patients using their AI chatbot were 3x more likely to start PrEP and nearly 2x more likely to stay on it.
- [Counsel Health](#) is a physician-led, AI-native virtual practice delivering asynchronous messaging-based care.

Appendix 2: Summary of regulatory recommendations

Recommendation	Current Barrier	Proposed Change	Regulation
Recognize asynchronous care as a billable modality	Store-and-forward reimbursement is limited to the Alaska and Hawaii demonstration programs	Amend to recognize asynchronous message-based clinical encounters as reimbursable nationwide; episode-based bundling (e.g., 7-day window) rather than per-message billing	42 CFR § 410.78(d)
Remove initial visit requirement for async care	Asynchronous follow-up requires a prior video or in-person encounter	Allow async-first encounters when identity verification, defined clinical question, documented clinical assessment, and treatment recommendation or escalation decision are in place	42 CFR § 410.78
Create an AI-augmented care modifier	E/M codes carry time requirements that penalize efficient AI-assisted encounters; no mechanism to bill AI-augmented visits at an appropriate rate	Establish modifier (similar to -93 audio-only) at reduced rate (e.g., 80%); tied to service delivered, not clinician minutes; requires documentation of AI-assisted components, escalation protocols, and clinician attestation	42 CFR § 414.22

Expand APCM for AI-enabled care	APCM requires synchronous initiating visit, comprehensive PCP attribution, and a single-practitioner billing model	Allow documented asynchronous onboarding; expand eligibility to condition-specific care management; recognize platforms with defined clinical governance and escalation protocols as billing entities; accommodate asynchronous follow-up for TCM's post-discharge visit requirement (CPT 99495, 99496)	CY 2025 PFS (G0556-G0558; 89 FR 97710)
Establish AI-autonomous billing codes	No billing mechanism for encounters where AI delivers clinical services directly with human escalation protocols	Create new service category for AI-autonomous clinical services; risk-tiered approach starting with lower-acuity, protocol-driven scenarios (UTI management, lab interpretation, medication refills)	42 CFR § 410.32 (or new section)
Pilot PMPM models for AI-enabled care	Always-on AI care does not fit discrete encounter billing logic	CMMI pilot of per-member-per-month model for AI-enabled primary care with straightforward attribution	CMMI authority